

Title Page

Indication-Specific Appendix (ISA 1) for LY4395089 in Crohn's Disease

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Master Protocol Title:

A Master Protocol for Phase 2, Randomized, Controlled Studies of Multiple Interventions for the Treatment of Adults with Moderately to Severely Active Ulcerative Colitis or Crohn's Disease

Master Protocol Number: J6Z-MC-IIBD

ISA Title: A Phase 2, Multicenter, Randomized, Open-Label, Active-Controlled Study to Investigate LY4395089/Mirikizumab Co-administration Compared with Mirikizumab in Adults with Moderately to Severely Active Crohn's Disease

ISA Number: J6Z-MC-CD01

Amendment Number: This is the initial protocol.

Compound: Mirikizumab (LY3074828) and LY4395089

ISA Brief Title: A Phase 2 Study to Investigate LY4395089/Mirikizumab Co-administration Compared with Mirikizumab in Adults with Moderately to Severely Active Crohn's Disease

Study Phase: 2

Sponsor Name: Eli Lilly and Company

Legal Registered Address: Eli Lilly and Company, Indianapolis, Indiana, USA 46285

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Medical Monitor Name and Contact Information will be provided separately.

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1. Protocol Summary

1.1. Synopsis

Master Protocol Title: A Master Protocol for Phase 2, Randomized, Controlled Studies of Multiple Interventions for the Treatment of Adults with Moderately to Severely Active Ulcerative Colitis or Crohn's Disease

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Regulatory Agency Identifier Number(s):

IND: 178821

EU trial number: 2025-524112-11-00

Rationale:

This is a Phase 2 proof-of-concept ISA designed to assess the potential efficacy and safety of LY4395089/mirikizumab co-administration in Crohn's disease (CD).

Objectives, Endpoints, and Estimands:

Objectives	Endpoints
Primary	
See synopsis for the IIBD master protocol.	
Secondary	
See synopsis for the IIBD master protocol.	

Overall Design:

This ISA will follow the design of the master protocol for Study J6Z-MC-IIBD (IIBD) for the treatment of participants with moderately to severely active CD using LY4395089/mirikizumab co-administration.

Brief Summary:

The purpose of this study is to measure the efficacy and safety of LY4395089/mirikizumab co-administration compared with mirikizumab in adult participants with moderately to severely active CD.

Study details for this ISA include:

- The study duration will be approximately 62 weeks.
- The treatment duration will be approximately 52 weeks.
- The visit frequency will be every 2, 4, 8, or 12 weeks, depending on the visit type and the study period.

This ISA will include 2 treatment groups:

- LY4395089/mirikizumab co-administration, and
- mirikizumab.

Study Population:

This study will enroll participants who have moderately to severely active CD as detailed in the IIBD master protocol.

Number of Participants:

See the IIBD master protocol synopsis.

Intervention Groups and Duration:

See the IIBD master protocol synopsis for details on mirikizumab intervention.

This table summarizes LY4395089 dosing and administration information.

Intervention Name	LY4395089
Dosage Level	6 mg QD
Route of Administration	Oral
Authorized as Defined by EU Clinical Trial Regulation	Not authorized in EU

Abbreviations: EU = European Union; QD = once daily.

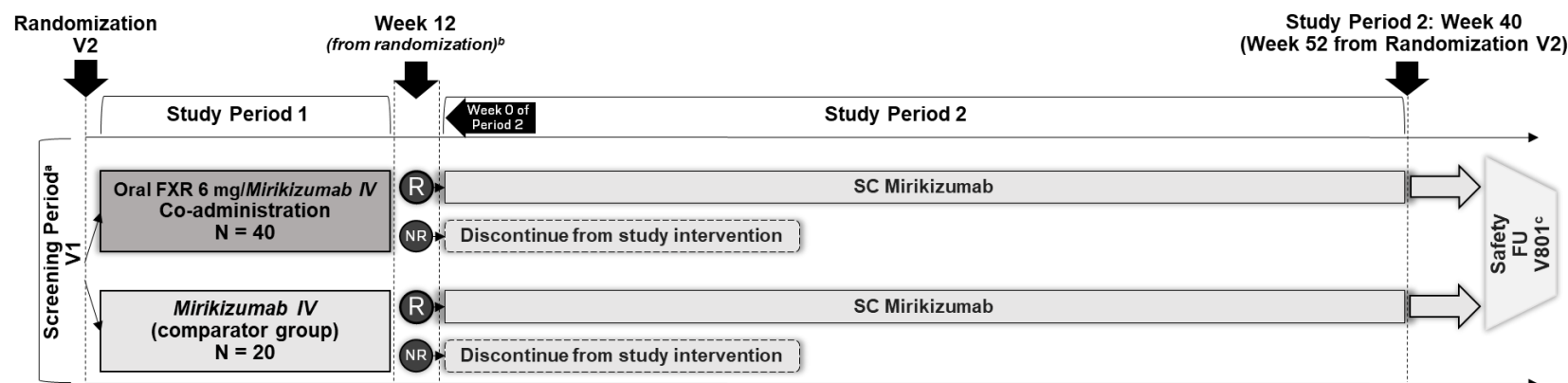
Ethical Considerations of Benefit/Risk:

Considering the impact of moderately to severely active CD on a patient's life, long-term burdensome complications, and therapeutic ceiling associated with advanced therapies, patients may benefit from complementary and additive effects of 2 different mechanisms of action from co-administration of mirikizumab and LY4395089.

At this time, no safety issues have been identified that would preclude the use of LY4395089 with mirikizumab co-administered to participants in this study. Considering the measures taken to minimize risks in the study along with the potential benefit of the co-administration, the benefit/risk balance for study participants is considered to be favorable.

Data Monitoring Committee: Yes (Internal Assessment Committee).

1.2. Schema



Abbreviations: CDAI = Crohn's Disease Activity Index; FU = follow-up; FXR = LY4395089; IV = intravenous; IWRS = interactive web-response system; N = number of participants; NR = nonresponder; R = responder; SC = subcutaneous.

- ^a Screening for the master protocol and the relevant ISA(s) will occur at the same time. If participants are eligible for more than 1 open ISA, they will be randomly assigned to an ISA and then to the treatment group using the IWRS.
- ^b As stated in Section 4.1 of the IIBD master protocol, responders at Week 12 of Study Period 1 will be defined as endoscopic response or clinical response (by CDAI). For detailed definitions, see Section 8.1 of the IIBD master protocol.
- ^c As stated in the SoA, the timing for the safety follow-up will be 8 weeks after the last dose.

1.3. Schedule of Activities (SoA)

1.3.1. Schedule of Activities for Screening and Study Period 1 of Study J6Z-MC-CD01

Note:

- For details about the transition of participants from Study Period 1 to Study Period 2, see the schema (Section 1.2) and the relevant notes in the SoA for Study Period 2 (Section 1.3.2).

Study J6Z-MC-CD01 SoA Table 1	Screening	Study Period 1					Comment
Visit number	1	2	3	4	5	6	
Weeks	-	0	2	4	8	12	
Visit interval in days	-42	-	±3	±3	±3	±7	
Visit detail		†				††	† Baseline/randomization †† Primary endpoint
Fasting visit		X				X	See fasting instructions in Section 8.2.5.
Complete informed consent	X						The ICFs must be signed before any protocol-specific tests or procedures are performed. See Section 10.1.3 of the IIBD master protocol for additional details.
Inclusion/exclusion criteria; confirmation of eligibility	X	X					The following confirmations are mandatory: - meeting <i>relevant</i> inclusion and exclusion criteria before endoscopy and bowel preparation (Section 8.1.1.3 of the IIBD master protocol), and - meeting <i>all</i> inclusion and exclusion criteria before randomization and administration of first dose of study intervention.
Demographics	X						Includes ethnicity (where permissible), year of birth, sex, and race.
Preexisting conditions and medical history, including relevant surgical history	X						Collect all ongoing conditions and relevant past surgeries and medical history.
Prespecified medical history	X						Indication and history of interest.
Prior treatments for indication	X						Includes all CD-specific therapies since date of diagnosis.
Concomitant medications	X	X	X	X	X	X	Collect data for all ongoing medications. See Section 6.9 of this ISA and the IIBD master protocol.
Adverse events (AEs)	X	X	X	X	X	X	Any events that occur after signing the informed consent are considered AEs as defined in Section 10.3 of the IIBD master protocol. Additional data are collected for certain AEs. For AESIs, see Section 8.3.3 of this ISA and the IIBD master protocol.
Substance use (alcohol, caffeine, tobacco)	X						
Physical evaluation							

Study J6Z-MC-CD01 SoA Table 1	Screening	Study Period 1					Comment
Visit number	1	2	3	4	5	6	
Weeks	-	0	2	4	8	12	
Visit interval in days	-42	-	±3	±3	±3	±7	
Visit detail		†				††	† Baseline/randomization †† Primary endpoint
Fasting visit		X				X	See fasting instructions in Section 8.2.5.
Height	X						
Weight	X	X		X	X	X	
Complete physical examination	X						The complete physical examination excludes pelvic, rectal, and breast examinations unless clinically indicated. Assess for TB risk factors and for signs and symptoms of active TB, including an assessment of peripheral lymph nodes (Sections 8.2.1 and 8.2.7 of the IIBD master protocol).
Symptom-directed physical assessment		X	X	X	X	X	Perform as shown in this schedule and as needed at any visit based on participant status and standard of care. Includes examination of eyes, throat, heart, lungs, abdomen, and visual examination of the skin (Section 8.2.1 of the IIBD master protocol). May be performed by qualified personnel per local regulations. Assess for TB risk factors and for signs and symptoms of active TB, including an assessment of peripheral lymph nodes (Sections 8.2.1 and 8.2.7 of the IIBD master protocol).
Vital signs	X	X	X	X	X	X	Include pulse rate, blood pressure, respiratory rate, and body temperature (Section 8.2.2 of the IIBD master protocol).
ECG, 12-lead (single, local)	X		X			X	See Section 8.2.3 of the IIBD master protocol.
X-ray of chest	X						Performed locally (posterior-anterior view and, if needed, a lateral view), interpreted, and reported by a radiologist or pulmonologist. Chest x-ray is not required if one was performed within 3 months (or 6 months in some jurisdictions) before Visit 1, and sufficient documentation exists for the TB evaluation (Sections 8.2.4 and 8.2.7 of the IIBD master protocol). A chest CT scan can be used instead of a chest x-ray if the chest CT scan was performed within the same time window (Section 8.2.4 of the IIBD master protocol).
Fistula evaluation		X		X	X	X	Evaluate for existing perianal fistulas (if present at baseline) and for any newly developed fistulas.
Crohn's Disease activity index (CDAI)	X*			X	X	X*	Patient-reported items of CDAI will be recorded daily via eDiary. Clinician reported items of CDAI will be recorded at the specified site visits. *Screening and Week 12 clinician-reported data are collected via an electronic outcomes instrument. For the remainder of the visits, these data will be entered directly into the CRF.

Study J6Z-MC-CD01 SoA Table 1	Screening	Study Period 1					Comment
Visit number	1	2	3	4	5	6	
Weeks	-	0	2	4	8	12	
Visit interval in days	-42	-	±3	±3	±3	±7	
Visit detail		†				††	† Baseline/randomization †† Primary endpoint
Fasting visit		X				X	See fasting instructions in Section 8.2.5.
							Collect clinical and laboratory components of CDAI (Section 8.1.2.1 of the IIBD master protocol) and assess participant eligibility at screening before colonoscopy and bowel preparation.
DXA scan		X*				X	*DXA scan must be completed at or within 7 days after V2. If rescan is needed and performed outside the 7 days, this will not be a protocol deviation.
Clinician-administered assessments (electronic)							
Review CDAI		X				X	
C-SSRS screening/baseline (Categories version)	X						AE collection should occur prior to the collection of the C-SSRS. Adapted for the assessment of ideation and behavior categories only.
Patient-reported outcomes (electronic)							
QIDS-SR16	X	X		X		X	
Participant daily diary (electronic)							The daily eDiary will be applicable only to Study Period 1 and ED, if ED occurs in Study Period 1. The daily eDiary for CD includes the following assessments (See 8.1.2.4 of the IIBD master protocol): • CDAI Stool Frequency • CDAI Abdominal Pain • CDAI General Well-Being • Bowel Movement Count
Daily eDiary training	X				X		Site personnel must ensure the participant is trained on daily eDiary completion before the endoscopy visits. Provide additional training as needed. For eDiary compliance requirements, see Section 8.1.2.1 of the IIBD master protocol.
Dispense daily eDiary	X						Site personnel must fully explain and confirm participant understanding of questionnaire before the device is provided to collect response.
Review daily eDiary	X	X	X	X	X	X	Participant eDiaries are to be completed daily by the participant at home and reviewed by study personnel. See Section 8.1.2.4 of the IIBD master protocol for more details about the eDiary.
Return daily eDiary		X*				X	*At baseline, collect diary from participants who were not eligible for randomization (screen failed).

Study J6Z-MC-CD01 SoA Table 1	Screening	Study Period 1					Comment
Visit number	1	2	3	4	5	6	
Weeks	-	0	2	4	8	12	
Visit interval in days	-42	-	±3	±3	±3	±7	
Visit detail		†				††	† Baseline/randomization †† Primary endpoint
Fasting visit		X				X	See fasting instructions in Section 8.2.5.
Participant education							
Lifestyle counseling (vitamin D/calcium counseling)		X		X	X		
Contraceptive counseling		X		X	X		
Laboratory tests and sample collections							In addition to the fasting visits for lipids testing, site personnel can make fasting recommendations per local guidelines for the procedures.
Hematology	X	X	X	X	X	X	
Clinical chemistry	X	X	X	X	X	X	
Coagulation	X						
Lipids (fasting)		X				X	At fasting visits.
Serum pregnancy	X						For IOCBP only (see Sections 8.2.6 and 10.4.1 of the IIBD master protocol).
Urine pregnancy (local)		X	X	X	X	X	For IOCBP, “negative” test required prior to dosing at marked visits. In Study Period 1, urine pregnancy tests will be done on site only. Additional pregnancy tests (urine or serum) should be performed at any time during the study if a menstrual period is missed, or there is clinical suspicion of pregnancy, or as required by local law or regulation (Sections 8.2.6 and 10.4.1 of the IIBD master protocol).
Follicle-stimulating hormone (FSH)	X	X*					*Optional; perform as needed to confirm postmenopausal status in individuals AFAB. See Sections 8.2.6 and 10.4.1 of the IIBD master protocol.
Parathyroid hormone (PTH)		X				X	
Urinalysis	X						
HIV testing	X						
Hepatitis B virus (HBV) screening tests	X						Screening includes HBsAg and anti-HBc. If HBsAg is negative and anti-HBc is positive, further testing for HBV DNA is required (Section 8.2.8 of the IIBD master protocol).
HBV DNA	X				X		For participants who are anti-HBc positive at screening (Section 8.2.8 of the IIBD master protocol).
Hepatitis C virus (HCV) screening tests	X						If HCV Ab test is positive, perform HCV RNA test (Section 8.2.9 of the IIBD master protocol).
Tuberculosis (TB) Test	X						Participants who had a TST will return 48 to 72 hours after placement to have their test results read. PPD test does not need to be read at the site but must be read by a trained medical professional, and the result must be presented to the site personnel prior to participant’s randomization (Section 10.7 of the IIBD master protocol).

Study J6Z-MC-CD01 SoA Table 1	Screening	Study Period 1					Comment
Visit number	1	2	3	4	5	6	
Weeks	-	0	2	4	8	12	
Visit interval in days	-42	-	±3	±3	±3	±7	
Visit detail		†				††	† Baseline/randomization †† Primary endpoint
Fasting visit		X				X	See fasting instructions in Section 8.2.5.
C-reactive protein, high sensitivity (hsCRP)	X	X*	X	X	X	X	*Collect only if not collected before Visit 2.
Synthetic glucocorticoid testing	X	X	X	X	X	X	
Serum 25-hydroxy vitamin D	X	X				X	
eGFR (CKD-EPI) calculated using creatinine	X	X		X		X	
Urinary albumin/creatinine ratio (UACR)		X				X	
IgG, IgA, IgM (quantitative immunoglobulins)		X		X		X	
Calcium (urine)		X				X	
P1NP		X				X	
CTX-1		X				X	
Bone-specific ALP		X				X	
FGF19		X	X	X	X	X	
7alphaC4 bile acid		X	X	X	X	X	
Pharmacokinetics (PK) samples for LY4395089				X*	X**		* At Visit 4, collect predose steady state. ** At Visit 5, collect post dose as close as possible to 4 hours post LY4395089 administration. Note: For LY4395089/mirikizumab co-administration treatment group only. See Section 8.4 for PK sample instructions.
Stool tests and sample collections							
Dispense stool collection kit	X		X	X	X		Dispense as needed.
<i>C. difficile</i> testing	X		X*				*Additional <i>C. difficile</i> testing is allowed throughout the study at the investigator's discretion.
Stool culture	X		X*				Other local stool testing, for example, for ova and parasites, is allowed at the investigator's discretion. *Additional stool culture testing is allowed throughout the study at the investigator's discretion.
Fecal calprotectin	X	X*		X	X	X	Stool samples should be collected before endoscopy preparation, if applicable.

Study J6Z-MC-CD01 SoA Table 1	Screening	Study Period 1					Comment
Visit number	1	2	3	4	5	6	
Weeks	-	0	2	4	8	12	
Visit interval in days	-42	-	±3	±3	±3	±7	
Visit detail		†				††	† Baseline/randomization †† Primary endpoint
Fasting visit		X				X	See fasting instructions in Section 8.2.5.
							*Collect only if not collected before Visit 2.
Stored samples							
Genetics sample		X					Sample can be obtained at or after the specified visit. If country regulations allow.
Exploratory biomarker, blood		X		X	X	X	
Exploratory biomarkers, stool supernatant	X	X*		X	X	X	Supernatant from the same stool sample used for fecal calprotectin testing. *Collect only if not collected before Visit 2.
Exploratory biomarkers, colonic tissue	X					X	Obtained from endoscopic biopsy (Section 8.1.1.3 of the IIBD master protocol).
Endoscopy							
Pre-endoscopy instructions and reminders	X				X		Includes daily eDiary compliance, dates, and bowel preparation/procedure.
Endoscopy with biopsy	X					X*	An endoscopy will be performed on all participants during screening within 21 days before randomization. For types of endoscopy allowed for CD, see Section 8.1.1.3 of the IIBD master protocol. Second endoscopy is allowed if inadequate preparation or equipment issues. *At Week 12 of Study Period 1, the endoscopy should be scheduled sufficiently early within the visit window to ensure the results are available to confirm the participant's response status before starting Study Period 2.
Randomization and dosing related activities							
Register visit with IWRS	X	X		X	X	X	
Randomization or assignment to ISA via IWRS		X					If a participant is eligible for more than 1 open ISA, they will be randomly assigned to an ISA.
Randomization to study intervention via IWRS		X					
Administer study intervention - mirikizumab IV		X		X	X		
Dispense LY4395089		X		X	X		
Return and accountability of LY4395089				X	X	X	

Study J6Z-MC-CD01 SoA Table 1	Screening	Study Period 1					Comment
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Visit interval in days	-42	-	±3	±3	±3	±7	
Visit detail		†				††	† Baseline/randomization †† Primary endpoint
Fasting visit		X				X	See fasting instructions in Section 8.2.5 .
Dispense ancillary supplies to participant		X					Dispense as needed.
Assess study intervention compliance				X	X	X	

1.3.2. Schedule of Activities for Study Period 2 of Study J6Z-MC-CD01

Notes:

For details about the transition of participants from Study Period 1 to Study Period 2, see the schema (Section 1.2).

- Participants who received LY/mirikizumab co-administration *or* mirikizumab in Study Period 1 and who are considered responders at Week 12 of Study Period 1 will receive mirikizumab SC starting at Week 0 of Study Period 2. For definition of responders, see Sections 4.1 and 8.1 of the IIBD master protocol.
- For responders to mirikizumab or LY/mirikizumab co-administration treatment, Visit 6 and Visit 7 will occur on the same day and the Visit 6 procedures are not repeated at Visit 7 (excluding “Register visit with IWRS”).
- Visit 8 and Visit 10 of the IIBD master protocol SoA do not apply to this ISA.

Study J6Z-MC-CD01 SoA Table 2	Study Period 2					Comment
Visit number	7	9	11	12	13	
Weeks	0	8	16	28	40	
Visit interval in days	-	±3	±7	±7	±7	
Visit detail	†					† V7 will be conducted on same day as V6
Concomitant medications		X	X	X	X	Collect data for all ongoing medications. See Section 6.9 of this ISA and the IIBD master protocol.
Adverse events (AEs)		X	X	X	X	Any events that occur after signing the informed consent are considered AEs as defined in Section 10.3 of the IIBD master protocol. Additional data are collected for certain AEs. See Section 8.3.3 of this ISA and the IIBD master protocol for AESIs.
Physical evaluation						
Weight				X	X	
Symptom-directed physical assessment		X	X	X	X	Perform as shown in this schedule and as needed at any visit based on participant status and standard of care. Includes examination of eyes, throat, heart, lungs, abdomen, and visual examination of the skin (Section 8.2.1 of the IIBD master protocol). May be performed by qualified personnel per local regulations. Assess for TB risk factors and for signs and symptoms of active TB, including an assessment of peripheral lymph nodes (Sections 8.2.1 and 8.2.7 of the IIBD master protocol).
Vital signs		X	X	X	X	Include pulse rate, blood pressure, respiratory rate, and body temperature (Section 8.2.2 of the IIBD master protocol).
Laboratory tests and sample collections						Site personnel can make fasting recommendations per local guidelines for the procedures.
Hematology		X	X	X	X	
Clinical chemistry		X	X	X	X	
Urine pregnancy (local)		X	X	X	X	For IOCBP, “negative” test required prior to dosing at marked visits. In Study Period 2, urine pregnancy tests can be done on-site or at home. Additional pregnancy tests (urine or serum) should be

Study J6Z-MC-CD01 SoA Table 2	Study Period 2					Comment
Visit number	7	9	11	12	13	
Weeks	0	8	16	28	40	
Visit interval in days	-	±3	±7	±7	±7	
Visit detail	†					† V7 will be conducted on same day as V6
						performed at any time during the study if a menstrual period is missed, or there is clinical suspicion of pregnancy or as required by local law or regulation (Sections 10.4.1 and 10.4.1 of the IIBD master protocol).
Follicle-stimulating hormone (FSH)		X*				*Optional per investigator's discretion; perform as needed to confirm postmenopausal status in individuals AFAB. See Section 8.2.6 and 10.4.1 of the IIBD master protocol.
HBV DNA		X	X	X	X	For participants who are anti-HBc positive at screening (Section 8.2.8 of the IIBD master protocol).
C-reactive protein, high sensitivity (hsCRP)		X	X		X	
Parathyroid hormone (PTH)					X	
Serum 25-hydroxy vitamin D					X	
P1NP					X	
CTX-1					X	
Bone-specific ALP					X	
Stool tests and sample collections						
Dispense stool collection kit	X			X		Dispense as needed.
<i>C. difficile</i> testing		X*				*Additional <i>C. difficile</i> testing is allowed throughout the study at the investigator's discretion.
Stool culture		X*				Other local stool testing, for example, for ova and parasites, is allowed at the investigator's discretion. *Additional stool culture testing is allowed throughout the study at the investigator's discretion.
Fecal calprotectin		X			X	
Participant dosing diary for Mirikizumab (paper)						
Dosing diary training	X					
Dispense dosing diary	X	X	X	X		
Dosing diary return		X	X	X	X	
Dosing diary review		X	X	X	X	
Participant education						
Contraceptive counselling	X					
Randomization and dosing related activities						
Register visit with IWRS	X	X	X	X	X	
Injection training - mirikizumab SC	X	X*				*Repeat as needed See Section 6.1.4.1 for mirikizumab injection training details.

Study J6Z-MC-CD01 SoA Table 2	Study Period 2					Comment
Visit number	7	9	11	12	13	
Weeks	0	8	16	28	40	
Visit interval in days	-	±3	±7	±7	±7	
Visit detail	†					† V7 will be conducted on same day as V6
Dispense study intervention - mirikizumab (for at-home dosing)	X	X	X	X		
Dispense ancillary supplies to participant	X*					*Dispense as needed.
Participant returns study intervention and ancillary supplies		X	X	X	X	
Assess study intervention compliance		X	X	X	X	

1.3.3. Schedule of Activities for Unscheduled Visits, Early Discontinuation, and Safety Follow-Up of Study J6Z-MC-CD01

Study J6Z-MC-CD01 SoA Table 3	UV	ED	Safety Follow-Up	Comment
Visit number	-	-	801	
Weeks	-	-	†	† The timing for the safety follow-up will occur 8 weeks after the last dose of study drug.
Visit interval in days	-	-	±7	
Visit detail				
Concomitant medications	X	X	X	Collect data for all ongoing medications. See Section 6.9 of this ISA and the IIBD master protocol.
Adverse events (AEs)	X	X	X	Any events that occur after signing the informed consent are considered AEs as defined in Section 10.3 of the IIBD master protocol. Additional data are collected for certain AEs. For AESIs, see Section 8.3.3 of this ISA and the IIBD master protocol.
Physical evaluation				
Weight	X	X		
Symptom-directed physical assessment	X	X	X	Perform as shown in this schedule and as needed at any visit based on participant status and standard of care. Includes examination of eyes, throat, heart, lungs, abdomen, and visual examination of the skin (Section 8.2.1 of the IIBD master protocol). May be performed by qualified personnel per local regulations. Assess for TB risk factors and for signs and symptoms of active TB, including an assessment of peripheral lymph nodes (Sections 8.2.1 and 8.2.7 of the IIBD master protocol).
ECG		X		Needed at ED visit only if prior to Week 12.
Vital signs	X	X	X	Include pulse rate, blood pressure, respiratory rate, and body temperature (Section 8.2.2 of the IIBD master protocol).
Fistula evaluation		X		At ED, the fistula evaluation is needed only when the ED is during Study Period 1. Evaluate for existing perianal fistulas (if present at baseline) and for any newly developed fistulas.
CDAI		X		At ED, the CDAI assessment is needed only when the ED is during Study Period 1.
DXA scan		X		At ED, the DXA scan is needed only when the ED is during Study Period 1.
Participant daily diary (electronic)				The daily eDiary will be applicable only to Study Period 1 and ED, if ED occurs in Study Period 1. The daily eDiary for CD includes the following assessments (see Section 8.1.2.4 of the IIBD master protocol): • CDAI Stool Frequency • CDAI Abdominal Pain • CDAI General Well-Being • Bowel Movement Count
Review daily eDiary		X		
Return daily eDiary		X		
Laboratory tests and sample collections				
Hematology		X	X	
Clinical chemistry		X	X	

Study J6Z-MC-CD01 SoA Table 3	UV	ED	Safety Follow-Up	Comment
Visit number	-	-	801	
Weeks	-	-	†	† The timing for the safety follow-up will occur 8 weeks after the last dose of study drug.
Visit interval in days	-	-	±7	
Visit detail				
Lipids (fasting)		X		Needed at ED visit only if prior to Week 12. Non-fasting lipids at ED will not be considered a protocol deviations, but fasting state should be recorded at time of collection.
Urine pregnancy (local)		X	X	For IOCBP only (see Sections 8.2.6 and 10.4.1 of the IIBD master protocol).
Follicle-stimulating hormone (FSH)		X*		*Optional per investigator's discretion; perform as needed to confirm postmenopausal status in individuals AFAB. See Sections 8.2.6 and 10.4.1 of the IIBD master protocol.
Parathyroid hormone (PTH)		X		
HBV DNA		X		For participants who are anti-HBc positive at screening (Section 8.2.8 of the IIBD master protocol). At ED, this test is not required if done within the last 8 weeks.
C-reactive protein, high sensitivity (hsCRP)		X		
Serum 25-hydroxy vitamin D		X		
eGFR (CKD-EPI) calculated using creatinine		X		
Urinary Albumin/Creatinine Ratio (UACR)		X		Needed at ED visit only if prior to Week 12.
IgG, IgA, IgM (quantitative immunoglobulins)		X		Needed at ED visit only if prior to Week 12.
Calcium (urine)		X		Needed at ED visit only if prior to Week 12.
P1NP		X		
CTX-1		X		
Bone-specific ALP		X		
Stool tests and sample collections				
Fecal calprotectin		X		
Stored samples				
Exploratory biomarkers, blood		X		Needed at ED visit only if prior to Week 12.
Exploratory biomarkers, stool supernatant		X		Supernatant from the same stool sample used for fecal calprotectin testing. Needed at ED visit only if prior to Week 12.
Visit registration activities				
Register visit with IWRS		X		
Participant returns study intervention and ancillary supplies		X		

Study J6Z-MC-CD01 SoA Table 3	UV	ED	Safety Follow-Up	Comment
Visit number	-	-	801	
Weeks	-	-	†	† The timing for the safety follow-up will occur 8 weeks after the last dose of study drug.
Visit interval in days	-	-	±7	
Visit detail				
Assess study intervention compliance		X		

Abbreviations: Ab = antibody; AE = adverse event; AESI = adverse event of special interest; AFAB = assigned female at birth; anti-HBc = hepatitis B core antibody; ALP = alkaline phosphatase; CD = Crohn's disease; CDAI = Crohn's Disease Activity Index; CRF = case report form; C-SSRS = Columbia Suicide-Severity Rating Scale; CT = computed tomography; CTX-1 = C-terminal telopeptide; DXA = dual-energy x-ray absorptiometry; ECG = electrocardiogram; ED = early discontinuation; eDiary = electronic diary; eGFR (CKD-EPI) = estimated glomerular filtration rate using the CKD-EPI equations; FGF19 = fibroblast growth factor 19; FSH = follicle-stimulating hormone; HBsAg = hepatitis B surface antigen; HBV = hepatitis B virus; HCV = hepatitis C virus; HIV = human immunodeficiency virus; hsCRP = C-reactive protein, high sensitivity; ICF = informed consent form; Ig = immunoglobulin; IOCBP = individuals of childbearing potential; IV = intravenous; IWRS = interactive web-response system; LY = study intervention other than an active comparator; P1NP = procollagen type I N-propeptide; PK = pharmacokinetics; PPD = purified-protein derivative; PTH = parathyroid hormone; QIDS-SR16 = Quick Inventory for Depressive Symptomatology - Self Report 16 items; SC = subcutaneous; SoA = schedule of activities; TB = tuberculosis; TST = tuberculin skin test; UACR = urinary albumin/creatinine ratio; UV = unscheduled visit; V = visit.

2. Introduction

2.1. Study Rationale

This is a Phase 2 proof-of-concept ISA designed to assess the potential efficacy and safety of LY4395089/mirikizumab co-administration in CD.

2.2. Background

2.2.1. Overview of the Master Protocol

See the IIBD Master protocol.

2.2.2. Overview of Ulcerative Colitis and Crohn's Disease

2.2.2.1. Ulcerative Colitis

Not applicable to this ISA.

2.2.2.2. Crohn's Disease

See the IIBD master protocol.

2.2.3. Study Intervention (LY4395089) Other than Mirikizumab

FXR is a nuclear hormone receptor expressed at high levels in the liver and intestinal tract (Forman et al. 1995; Higashiyama et al. 2008). FXR has been shown to be an important factor in the regulation and interaction of many pathways critical to metabolism and inflammation (Vavassori et al. 2009). FXR-null mice exhibit a dysregulated colonic inflammatory phenotype that is further exacerbated when such mice were exposed to colitis-inducing TNBS, the extent of which was significantly greater than that observed in TNBS-treated wild-type mice (Vavassori et al. 2009). In cholestatic rat models, FXR agonism has been shown to improve gut barrier function, enhance expression of tight junction proteins claudin-1 and occludin, reduce bacterial translocation, and mitigate intestinal inflammation (Verbeke et al. 2015).

In conditions like biliary obstruction, FXR signaling is disrupted, leading to increased intestinal permeability, inflammation, and epithelial apoptosis (Yan et al. 2022). Treatment with FXR agonists such as obeticholic acid has been shown to restore FXR signaling, reduce intestinal inflammation, and improve barrier function by modulating bile acid transporters and promoting epithelial cell proliferation (Yan et al. 2022). Furthermore, FXR activation influences the gut microbiota composition, enhancing beneficial bacteria and reducing harmful species, which contributes to improved intestinal and hepatic health. Importantly, FXR also plays a protective role against liver fibrosis by regulating bile acid synthesis and reducing inflammatory signaling, suggesting its therapeutic potential in fibrotic diseases associated with gut-liver axis dysfunction (Yan et al. 2022).

LY4395089 is an optimized FXR agonist with a non-bile acid chemical structure and sustained FXR engagement. LY4395089 has been evaluated in preclinical models of intestinal colitis. In vitro assays with 3D human organoid cultures derived from CD patients, LY4395089

demonstrated enhanced intestinal barrier integrity, and reduced inflammation and fibrotic markers (Mon et al. 2024). In the T-cell adoptive transfer mouse model of inflammatory bowel disease LY4395089 treatment was associated with improved colitis inflammation and colon histopathology (Liu et al. 2021). Additionally, LY4395089 has been studied in a first-in-human single-/multiple-dose Phase 1 Study MET642-101 (J6P-MC-NFAB [NFAB]) and Phase 2 Study MET642-201 (J6P-MC-NFAA [NFAA]) in participants with MASH.

2.2.4. Mirikizumab as an Active Comparator

See the IIBD master protocol.

2.3. Benefit/Risk Assessment

Detailed information about the known and expected benefits and risks and reasonably expected AEs of mirikizumab and LY4395089 may be found in the relevant IBs. In addition, for mirikizumab, see prescribing information.

2.3.1. Risk Assessment

2.3.1.1. Study Intervention

Potential risks of LY4395089

Transient increases in liver enzymes (ALT and AST) above the ULN were observed in Phase 1 and Phase 2 studies. None of the ALT or AST increases in either study were accompanied by clinically significant changes in ALP, GGT, or total bilirubin, and there were no clinical findings suggestive of hepatic injury. There were no significant changes in LDL-C. Decreases in HDL-C were observed with LY4395089 administration in either Phase 1 or Phase 2 studies, consistent with the known effects of this class of medications. Risks include pruritus (mild and manageable), as well as potential effects on bone health (osteopenia and fractures). Although no cases of photosensitivity have been reported in clinical studies with LY4395089 to date, preclinical ultraviolet absorbance testing indicates a potential risk for phototoxicity. There is no anticipated increased risk of infection for LY4395089.

For additional details on risks of LY4395089, see the IB.

Potential risks with active comparator: mirikizumab

For details about the adverse drug reactions and special warnings and precautions for mirikizumab use, see the IB and prescribing information.

Potential risks of LY4395089/mirikizumab co-administration

When LY4395089/mirikizumab is co-administered, no pharmacologic interaction is expected. Elevations of aminotransferases have been reported in patients receiving mirikizumab and in patients receiving LY4395089. Detailed instructions for hepatic monitoring, evaluation, and criteria for drug interruption or discontinuation are described in Section 8.2.10 of the IIBD master protocol.

To address these potential risks, hepatic function, bone safety, and tolerability will be monitored throughout the study.

2.3.1.2. Study Procedures

See the IIBD master protocol.

The risk associated with DXA scan is low, with a radiation exposure (effective dose) of about 5 μ Sv or 0.005 mSv. The typical level of background radiation to which the general population is exposed, not including radiation due to medical procedures, has been estimated to be about 2.5 mSv/year (Lewiecki et al. 2016).

2.3.1.3. Management of Risks

The potential risks will be proactively monitored. The following sections in this ISA address known potential risks associated with ISA-specific study procedures or LY4395089:

- [2.3.1.2](#)
- [5.1](#) and [5.2](#)
- [7.1](#)
- [8.2](#), and
- [8.3](#).

The following sections in the IIBD master protocol address known potential risks associated with study procedures, disease state, or mirikizumab.

- [2.3.1.2](#)
- [8.1.1.3](#)
- [5.1](#) and [5.2](#)
- [7.1](#) and [7.2](#)
- [8.2](#), and
- [8.3](#).

2.3.2. Benefit Assessment

2.3.2.1. Benefits of Study Intervention (LY4395089) Other than Active Comparator

Drawing on preclinical data for LY4395089 and considering the physiological role of FXR within the intestines, treatment with LY4395089 may offer potential benefits to study participants with CD. LY4395089 enhances intestinal barrier integrity, reduces proinflammatory cytokines, and mitigates fibrotic activity in CD patient primary cell 3D models (Mon et al. 2024).

2.3.2.2. Benefits of Active Comparator: Mirikizumab

See the IIBD master protocol.

2.3.2.3. Benefits of LY4395089/Mirikizumab Co-administration

The co-administration of LY4395089 (a potent, selective, FXR agonist) and mirikizumab (an IL-23p19 monoclonal antibody) may provide a therapeutic benefit to patients with moderately to severely active CD. While no data are currently available for LY4395089/mirikizumab co-administration in human participants, mirikizumab, approved for CD in multiple geographies, targets the IL-23 pathway to suppress systemic immune activation and has shown durable clinical and endoscopic improvements (Ferrante et al. 2024). Mechanistically, LY4395089 acts

upstream of mirikizumab by modulating innate immune responses and epithelial barrier function, thereby reducing bacterial translocation and cytokine release. By targeting 2 mechanisms of action, the co-administration of LY4395089 and mirikizumab may provide additional therapeutic benefit for participants with moderately to severely active CD.

2.3.3. Overall Benefit Risk Conclusion

Taking into account the measures to minimize risk to participants in this study, the potential risks identified in association with LY4395089 and mirikizumab are justified by the anticipated benefits that may be afforded to participants with CD.

3. Objectives, Endpoints, and Estimands

For definitions of CD-specific endpoints, see Section 8.1 of the IIBD master protocol.

Objectives	Endpoints
Primary	
See the IIBD master protocol.	
Secondary	
See the IIBD master protocol.	
Exploratory	
To evaluate the effect of LY4395089 on bone health and lipid metabolism in participants with CD	Change from baseline at Week 12 of Study Period 1 and Week 40 of Study Period 2 in the following bone health biomarkers: • CTX-1 • PTH • P1NP • 25-OH-vitamin D • Bone-specific alkaline phosphatase Change from baseline at Week 12 of Study Period 1 for bone mineral density Change from baseline at Week 12 of Study Period 1 of • Total cholesterol • TG • Non-HDL cholesterol • LDL • HDL
To evaluate the pharmacodynamic effects of LY4395089 in participants with CD	Change from baseline at Week 12 of Study Period 1 in the following biomarkers: • 7AlphaC4 • FGF19

Abbreviations: CD = Crohn's disease; CTX-1 = C-terminal telopeptide; FGF19 = fibroblast growth factor 19; HDL = high-density lipoprotein; LDL = low-density lipoprotein; P1NP = procollagen type 1 propeptide; PTH = parathyroid hormone; TG = triglyceride.

Primary estimand/coprimary estimand

See the IIBD master protocol.

Secondary estimand(s)

See the IIBD master protocol.

4. Study Design

4.1. Overall Design

Study design overview

Study J6Z-MC-CD01 (CD01) is a Phase 2, randomized, open-label, active-controlled study to investigate FXR314 agonist LY4395089 co-administration with mirikizumab in adults with moderately to severely active CD.

The study design of this ISA will be governed by the IIBD master protocol.

Duration and study periods for this ISA

The duration and study periods for this ISA will be governed by the IIBD master protocol. A schematic of the design specific to this ISA is presented in Section 1.2. Assessments, sample collections, and study drug administrations for this ISA will occur at the visits shown in the SoA (Section 1.3).

Treatment groups

This table summarizes the treatment groups under this ISA.

Study Interventions in Study Period 1	Study Intervention in Study Period 2
LY4395089/mirikizumab co-administration	Mirikizumab
Active comparator: mirikizumab	

Randomization ratios and stratification factors

See the IIBD master protocol.

Transition from Study Period 1 to Study Period 2

See the IIBD master protocol and study schema (Section 1.2).

4.2. Scientific Rationale for Study Design

See the IIBD master protocol.

4.2.1. Patient Input into Design

See the IIBD master protocol.

4.3. Justification for Dose

4.3.1. Justification for Dose of Study Intervention (LY4395089) Other than Active Comparator

The dose level of LY4395089 was selected based on clinical experience, safety margins, PD activity, and considerations for co-administration.

The 6 mg dose represents the highest dose of LY4395089 with participants having at least 12 weeks of exposure in a prior study (Study NFAA). This dose was well tolerated, with no significant hepatotoxicity or lipid imbalance observed. Additionally, at a 6 mg dose, LY4395089 elicited a PD response, supporting potential efficacy.

4.3.2. Justification for Doses of Mirikizumab

Mirikizumab will be dosed and administered per approved product labeling for adults with CD. See the IIBD master protocol for additional details.

4.3.3. Justification for LY4395089/Mirikizumab Co-administration

No direct pharmacologic interactions between LY4395089 and mirikizumab co-administration are expected. Participants will be monitored for known risks for both compounds, particularly where those risk profiles overlap.

4.4. End of Study Definition

See the IIBD master protocol.

5. Study Population

Participant eligibility for enrollment in Study CD01 is based on the criteria listed in the IIBD master protocol and the criteria for ISA CD01.

5.1. Inclusion Criteria

Participants are eligible to be included in this ISA only if all of the inclusion criteria in the IIBD master protocol, except the UC-specific criteria, and the following ISA-specific criteria apply:

Sex assigned at birth and contraceptive/barrier requirements

[1001] See the IIBD master protocol for details, including

- AMAB and AFAB participation in the study, including IOCBP and INOCBP, and
- contraception criteria for mirikizumab.

See Section 10.4 of this ISA for the contraception criteria for LY4395089/mirikizumab co-administration.

Prior medication experience

[1002] Participants taking GLP-1 RAs, GLP-1/GIP RAs, GLP-1/Gcg RAs, GLP-1/GIP/Gcg RAs, or similar medications for approved indications will be permitted to enroll provided they are on a stable dose at the time of screening (no anticipated dose escalations).

5.2. Exclusion Criteria

Participants are excluded from the study if any of the exclusion criteria in the IIBD master protocol, except the UC-specific criteria apply, or if any of the following criteria apply:

Medical conditions

Renal

[1003] Have an eGFR <90 mL/min/1.73 m² determined by the central laboratory during screening (Visit 1).

Hepatic

[1004] Have a hepatic disease or hepatic impairment, defined as any of the following:

- cirrhosis, primary biliary cholangitis, viral hepatitis, autoimmune hepatitis, or signs or symptoms of any other liver disease (exception: metabolic dysfunction-associated steatotic liver disease [MASLD])
- any degree of hepatic impairment as defined by the Child-Pugh classification (Child-Pugh A, B, or C), regardless of etiology, or

Other medical conditions

- [1005] Have a history of osteoporosis, osteomalacia, or presence of untreated severe vitamin D deficiency (serum 25-hydroxy-vitamin D ≤ 5 ng/mL).
- [1006] Have a history of fracture, bone surgery (for example, hardware placement, joint replacement, bone grafting, or amputation), or clinically significant bone trauma within 8 weeks of screening.
- [1007] Have a history of Addison disease.
- [1008] Have a history of Paget's disease of the bone.
- [1009] Have a history of any other bone disease that affects bone metabolism (for example, osteopetrosis or osteogenesis imperfecta).
- [1010] Have had within the past 180 days before screening
- acute myocardial infarction
 - cerebrovascular accident (stroke)
 - hospitalization for unstable angina
 - hospitalization due to congestive heart failure, or
 - coronary revascularization.
- [1011] Have a history of chronic neutropenia.

Prior or concomitant therapy

- [1012] Have been treated with moderate or strong CYP3A4 inhibitors or inducers within 14 days or 5 half-lives (whichever is longer) prior to randomization, or plan to receive treatment with these CYP3A4 modulators during the study.
See Section 10.14 for a list of moderate and strong CYP3A4 inhibitors and inducers.
- [1013] Have been treated with CYP3A4 substrates with a narrow therapeutic index within 14 days or 5 half-lives (whichever is longer) prior to randomization, or plan to receive treatment with these CYP3A4 modulators during the study.
See Section 10.14 for a list of CYP3A4 substrates with a narrow therapeutic index.
- [1014] Have received any of the following bone active treatments within the time frames specified below, or plan to receive them during the study:
- oral bisphosphonate use
 - >3 months cumulative treatment in the past 2 years, OR
 - >1 month in the past year, OR
 - any use during the 3 months prior to screening
 - administration of intravenous zoledronic acid within the last 5 years
 - administration of intravenous bisphosphonate (excluding zoledronic acid), fluoride, or strontium within the past 2 years
 - parathyroid hormone (PTH), or PTH derivative (teriparatide, abaloparatide) in the past year, or
 - treatment with denosumab in the past year.
- [1015] Administration of any of the following treatments within 3 months of screening:

- any selective estrogen receptor modulator (SERM)
- tibolone
- anabolic steroids or testosterone
- systemic hormone replacement therapy with exception of confirmed hypogonadism treated with stable doses of testosterone replacement therapy for ≥ 2 years prior to screening
- calcitonin
- 25-hydroxy-vitamin D or 1,25-dihydroxy-vitamin D, that is, active vitamin D analogs; over the counter vitamin D supplements are acceptable
- the use of anticonvulsants known to affect bone metabolism (for example, phenytoin, carbamazepine, valproate) (benzodiazepines are permitted)
- heparin, or
- chronic systemic ketoconazole, androgens, adrenocorticotrophic hormone, cinacalcet, aluminum, lithium, protease inhibitors, and gonadotropic-releasing hormone agonists.

[1016] Have been treated with an FXR agonist.

5.3. Lifestyle Considerations

Meals and dietary restrictions

Participants treated with LY4395089 should refrain from consuming

- grapefruit or products from grapefruit
- Seville (bitter) oranges or products from Seville oranges, and
- star fruit or products from star fruit.

Contraceptive guidance for LY4395089

See Section [10.4](#).

Other lifestyle considerations

Participants will receive counseling regarding the recommended daily allowance of vitamin D and calcium according to the investigator's clinical judgment and in line with current dietary and clinical practice guidelines from the Institute of Medicine 2011 recommendations, considering the increased risk of deficiency in CD (Ross et al. 2011).

5.4. Screen Failures

See the IIBD master protocol.

5.4.1. Allowed Retesting of Screening Investigations

See the IIBD master protocol.

5.4.1.1. Screening Stool Testing

See the IIBD master protocol.

5.4.2. Rescreening of Individuals Who Fail Screening

See the IIBD master protocol.

5.5. Criteria for Temporarily Delaying Enrollment of a Participant

This section is not applicable to this study.

6. Study Intervention(s) and Concomitant Therapy

Study intervention is defined as any medicinal product(s) or medical device(s) intended to be administered to or used by a study participant according to the study protocol.

6.1. Study Intervention(s) Administered

Packaging and labeling

Study interventions will be supplied by the sponsor or its designee in accordance with current Good Manufacturing Practice. Study interventions will be labeled as appropriate for country requirements.

6.1.1. Study Intervention (LY4395089) Other than Mirikizumab

This table lists the LY4395089 dosing information for this study.

Intervention Name	LY4395089
Dosage Level	6 mg QD
Route of Administration	Oral
Authorized as Defined by EU Clinical Trial Regulation	Not authorized in EU

Abbreviations: EU = European Union; QD = once daily.

6.1.2. Active Comparator: Mirikizumab

See the IIBD master protocol.

6.1.2.1. Participant Monitoring and Observation After Administration of Mirikizumab

See the IIBD master protocol

6.1.3. Background Therapy

See the IIBD master protocol.

6.1.4. On-Site and Off-Site Administration of Study Intervention

LY4395089

At-home administration

Participants will receive LY4395089 for daily administration.

Participants should take study intervention at approximately the same time each day.

Missed doses

In the case of a missed dose, participants will be instructed not to take a “catch-up dose” but to resume dosing with the next regularly scheduled dose. Participants should then continue with the remaining doses as normal. A participant should never take a catch-up dose in this circumstance.

Mirikizumab infusion

See the IIBD master protocol.

Mirikizumab SC*Initial administration*

The initial administration of mirikizumab SC will be done on site

- as self-administration by participant or caregiver, after appropriate training and under supervision of study personnel, or
- by authorized study personnel.

Subsequent administration

Subsequent administration of mirikizumab SC can be done

- off-site (for example, at participant's home) by the participant or caregiver after the completion of training, or
- on-site, by authorized study personnel (per participant's preference).

6.1.4.1. Mirikizumab Self-administration Training and Dosing Diary**Training for mirikizumab self-administration**

Site personnel will train the participant and/or caregiver to administer mirikizumab as the SC injection using a prefilled syringe as specified in the SoA.

It is strongly recommended that the trained participant or caregiver administers the first dose of mirikizumab under supervision of the site personnel. Thereafter, study intervention should be administered by the participant or caregiver. If additional training is necessary, an injection may be administered by the participant and/or caregiver under the supervision of the site personnel. If the participant or their caregiver do not feel comfortable administering the injection, it can be done by the study personnel.

The investigator must document in the source documents that injection training has been provided verbally and in written form to the participant or caregiver at the first dispensing visit. Refresher training will be provided as needed throughout the study. Injection training is the responsibility of the investigator or other qualified staff.

Mirikizumab SC dosing diary

Dosing diary will be dispensed to each participant as indicated in the SoA (Section 1.3), for recording pertinent data about each injection. The forms must be completed for all SC doses, regardless of who administers.

6.1.5. Medical Devices, Investigational Devices, or Device Constituents

Not applicable for this study.

6.2. Preparation, Handling, Storage, and Accountability

See the IIBD master protocol.

6.3. Assignment to Study Intervention

See the IIBD master protocol.

6.4. Blinding

See the IIBD master protocol.

6.5. Study Intervention Compliance

When the participant and/or caregiver administers study intervention(s) at home, compliance with study interventions will be assessed at visits indicated in the SoA (Section 1.3). Compliance will be assessed by

- direct questioning
- counting of unused study intervention and/or empty bottles (LY4395089, co-administration arm, Period 1 only) or cartons (mirikizumab, both arms, Period 2 only) returned, and
- review of the mirikizumab SC dosing diary (both arms, Period 2 only).

Study intervention compliance will be documented in the source documents.

Participants will be instructed to return any unused study intervention supply at the times specified in the SoA.

Treatment compliance for each visit interval is defined as taking at least 75% of the required doses of study interventions.

In addition to the assessment of a participant's compliance with the study intervention administration, other aspects of compliance with the study treatments will be assessed at each visit based on

- the participant's adherence to the visit schedule
- completion of the mirikizumab dosing diary, and
- any other parameters the investigator considers necessary.

Participants considered to be poorly compliant with their study intervention and/or the study procedures will receive additional training and instruction, as required, and will be reminded of the importance of complying with the protocol.

6.6. Dose Modification

Dose modifications are not permitted in this study.

6.7. Continued Access to Study Intervention after the End of the Study

LY4395089 and mirikizumab will not be provided to participants after completion of the study.

6.8. Treatment of Overdose

6.8.1. Treatment of Overdose for Study Intervention (LY4395089) Other Than Mirikizumab

For this study, an overdose of LY4395089 is defined as any dose of LY4395089 greater than the specified number of doses assigned through randomization.

In the event of an overdose, the investigator should:

- contact the medical monitor immediately
- evaluate the participant to determine, in consultation with the medical monitor, whether study intervention should be interrupted or whether the dose should be reduced, and
- closely monitor the participant for any AE/SAE and laboratory abnormalities as medically appropriate.

6.8.2. Treatment of Overdose for Mirikizumab

See the IIBD master protocol.

6.9. Prior and Concomitant Therapy

6.9.1. General Considerations

See the IIBD master protocol.

6.9.2. Permitted Concomitant Medications

See the IIBD master protocol.

6.9.2.1. Corticosteroid Use and Taper

See the IIBD master protocol.

6.9.3. Prohibited Concomitant Medications

See the IIBD master protocol for the list of prohibited concomitant medications applicable to all ISAs, and Section 10.14 of this ISA for prohibited concomitant medications specific to this ISA.

6.9.4. Rescue Medicine

See the IIBD master protocol.

7. Discontinuation of Study Intervention and Participant Discontinuation/Withdrawal

See the IIBD master protocol.

7.1. Discontinuation of Study Intervention

7.1.1. Permanent Discontinuation of Study Intervention

See the IIBD master protocol for full list of reasons for permanent discontinuation.

The following discontinuation criteria are specific to this ISA.

A participant treated with LY4395089/mirikizumab co-administration in this ISA **must** be permanently discontinued from LY4395089 for occurrence of nontraumatic bone fracture confirmed by imaging and clinical evaluation (Section [7.1.2](#)).

7.1.2. Temporary Discontinuation of Study Intervention

See the IIBD master protocol for full list of reasons for temporary discontinuation of study intervention.

The following discontinuation criteria are specific to this ISA:

LY4395089 must be interrupted if a nontraumatic fracture is suspected (for example, new unexplained localized bone pain, swelling, tenderness, reduced mobility, pain during normal daily activities, visible deformity, or signs of vertebral compression such as sudden back pain or height loss) to ensure appropriate evaluation and management (see Section [7.1.1](#) for permanent discontinuation of LY4395089 related to nontraumatic bone fracture).

7.1.3. Hepatic Criteria for Study Drug Interruption or Discontinuation

See the IIBD master protocol.

7.1.4. QTc Stopping Criteria

Not applicable for this ISA.

7.2. Participant Discontinuation/Withdrawal from the Study

See the IIBD master protocol.

7.3. Lost to Follow-Up

See the IIBD master protocol.

8. Study Assessments and Procedures

See the IIBD master protocol.

8.1. Efficacy Assessments

See the IIBD master protocol for definitions of the CD-specific endpoints.

8.1.1. Primary Efficacy Assessments

See the IIBD master protocol.

8.1.1.1. mMS

Not applicable to this ISA.

8.1.1.2. Simple Endoscope Score for Crohn's Disease (SES-CD)

See the IIBD master protocol.

8.1.1.3. Endoscopy: Colonoscopy or Flexible Sigmoidoscopy

See the IIBD master protocol.

8.1.2. Secondary Efficacy Assessments

See the IIBD master protocol.

8.1.2.1. Crohn's Disease Activity Index (CDAI)

See the IIBD master protocol.

8.1.2.2. Colonic Biopsies

See the IIBD master protocol.

8.1.2.3. Histologic Scores

See the IIBD master protocol.

8.1.2.4. Patient-Reported Outcomes: Daily Electronic Diary

See the IIBD master protocol.

8.1.2.5. Inflammatory Biomarkers

See the IIBD master protocol.

8.2. Safety Assessments

See the IIBD master protocol for details on safety assessments. Planned time points for all safety assessments in this ISA are provided in the SoA (Section [1.3](#)).

8.2.1. Physical Examinations

See the IIBD master protocol.

8.2.2. Vital Signs

See the IIBD master protocol.

8.2.3. Electrocardiograms

See the IIBD master protocol.

8.2.4. Chest Imaging

See the IIBD master protocol.

8.2.5. Clinical Safety Laboratory Tests

See the IIBD master protocol. For a list of ISA-specific clinical laboratory tests, see Section [10.2](#).

Fasting visits

Participant should not eat or drink anything except water during the 12 hours before sample collection at the visits denoted as “Fasting visit” on the SoA (Section [1.3](#)). If participant is in a non-fasting state at the time of sample collection, samples should still be collected, and this will not be considered a protocol deviation. Study participants should follow any fasting recommendations for endoscopies per local or investigator recommendations.

8.2.6. Pregnancy Testing

See the IIBD master protocol.

8.2.7. Tuberculosis Testing and Monitoring

See the IIBD master protocol.

8.2.8. Hepatitis B Testing and Monitoring

See the IIBD master protocol.

8.2.9. Hepatitis C Testing and Monitoring

See the IIBD master protocol.

8.2.10. Hepatic Safety Monitoring, Evaluation, and Criteria for Study Drug Interruption or Discontinuation

See the IIBD master protocol.

8.2.10.1. Close Hepatic Monitoring

See the IIBD master protocol.

8.2.10.2. Comprehensive Hepatic Evaluation

See the IIBD master protocol.

8.2.10.3. Study Drug Interruption or Discontinuation

See the IIBD master protocol.

8.2.11. Depression, and Suicidal Ideation and Behavior Risk Monitoring

See the IIBD master protocol.

8.2.12. Hypersensitivity Reactions

See the IIBD master protocol.

8.2.13. Injection/Infusion Site Reactions (ISRs)

See the IIBD master protocol.

8.2.14. Dual-Energy X-Ray Absorptiometry (DXA Scan)

DXA scans will be performed to assess the potential effects of LY4395089 on BMD. Scans will be obtained at baseline (+7 days) and at week 12 (± 7 days) to evaluate BMD in the lumbar spine (L1 to L4) and hip region. Participants should avoid taking calcium supplements (including multivitamins containing calcium) on the day of the scan (within 24 hours). Because normative databases vary across DXA manufacturers, actual BMD values (g/cm^2) will be collected as the primary data. Clinical sites will perform scans according to institutional standards, with results expressed as BMD values and corresponding T-scores or Z-scores relative to the manufacturer's reference population. Whenever feasible, both timepoint assessments for an individual subject will be performed on the same instrument using the same software version to ensure consistency. All scans will be transferred to and analyzed by a central reader. Sites will be informed if participants have a T-score of less than -2.5, or a Z-score of less than -2.0.

8.3. Adverse Events, Serious Adverse Events, and Product Complaints

See the IIBD master protocol.

8.3.1. Timing and Mechanism for Collecting Events

See the IIBD master protocol.

8.3.2. Collection of Pregnancy Information**8.3.2.1. Participants Who Become Pregnant**

See the IIBD master protocol.

8.3.2.2. Participants with Partners Who Become Pregnant

See the IIBD master protocol.

8.3.3. Adverse Events of Special Interest

The following AESIs are specified in the IIBD master protocol and apply to all ISAs.

- infections, including opportunistic infections and serious infections
- hepatic events
- MACE, and
- malignancies.

The following AESIs will also apply to this ISA.

- Hepatotoxicity (including liver enzyme elevations and clinical manifestations)
- Bone metabolism disorders, fractures.

If any of the AESIs listed above are reported, sites will be prompted to collect additional details and data.

8.3.3.1. Infections, Including Opportunistic Infections and Serious Infections

See the IIBD master protocol.

8.3.3.2. Hepatic Events

See Section 8.2.10 of the IIBD master protocol.

8.3.3.3. Major Adverse Cardiovascular Events-3 (MACE-3)

See the IIBD master protocol.

8.3.3.4. Malignancies

See the IIBD master protocol.

8.3.3.5. Hepatotoxicity (Including Liver Enzyme Elevations and Clinical Manifestations)

In cases of abnormal hepatic laboratory or hepatic clinical signs or symptoms, close hepatic monitoring, or comprehensive hepatic evaluation should be initiated if they meet the criteria outlined in Section 8.2.10 of the IIBD master protocol. For the hepatic criteria for study intervention interruption or discontinuation, refer to Section 10.6 of the IIBD master protocol.

8.3.3.6. Bone Metabolism Disorders and Fractures

DXA scans will be performed to measure BMD in the lumbar spine and hip region.

Participants will be monitored for metabolic bone disorders through clinical laboratory testing including:

- bone specific ALP,
- CTX-1,
- P1NP,
- 25-hydroxy-vitamin D
- parathyroid hormone
- magnesium

- calcium, and
- phosphate.

Participants will be monitored for fractures. All AEs and SAEs will be recorded as described in Section 8.3.

8.4. Pharmacokinetics

Planned PK assessments

At visits and times specified in the SoA (Section 1.3), plasma samples will be collected from participants in the LY4395089/mirikizumab co-administration treatment group to determine concentrations of LY4395089. The actual time and date (24-hour clock time) of the prior dosing and of the sample collection must be recorded accurately on the appropriate forms.

Instructions for the collection and handling of plasma samples will be provided by the sponsor. Samples will be analyzed at a laboratory approved by the sponsor. Concentrations of LY4395089 will be determined using a validated assay.

Visit 4 PK sample

At Visit 4, participants will be asked to withhold taking their dose of LY4395089 prior to coming into the clinic for assessments. A PK sample will be taken prior to the participant taking their dose of LY4395089 at the clinic. Mirikizumab doses can be administered as scheduled.

Visit 5 PK sample

At Visit 5, participants should take their dose of LY4395089 prior to coming into the clinic for assessments. A PK sample should be taken as close to 4 hours post LY4395089 administration as possible. If the participant did not take the LY4395089 dose prior to the clinic visit, participants should take their daily dose of LY4395089 upon arrival and a PK sample collected as close to 4 hours post dose as is reasonable during planned visit activities. If participants are running out of doses prior to their visit (due to an unexpected circumstance), participants need to save a dose for the day of Visit 5 and take that dose prior to the clinic visit. Mirikizumab doses can be administered as scheduled.

PK assessments in the case of hypersensitivity or ISR

See the IIBD master protocol.

Sample retention

See Section 10.1.12 of the IIBD master protocol.

8.5. Pharmacodynamics

The named PD biomarkers to be measured include

- 7AlphaC4, and
- FGF19.

The actual time and date (24-hour clock time) of the prior dosing and of the sample collection must be recorded accurately on the appropriate forms.

Sample collection information

The PD samples will be collected at the times specified in the SoA (Section 1.3) and as detailed in Section 10.2 and the laboratory manual.

Sample use

See Section 3 and the SAP.

8.6. Genetics

See the IIBD master protocol.

8.7. Biomarkers**Named biomarkers**

See Section 8.5.

Exploratory biomarkers***Sample collection information***

Blood, stool supernatant, urine, and tissue samples will be collected at times specified in the SoA (Section 1.3) and as detailed in Section 10.2 and the laboratory manual.

Sample use

See the IIBD master protocol.

Confidentiality of samples

See the IIBD master protocol.

Sample retention

See Section 10.1.12 of the IIBD master protocol.

8.8. Immunogenicity Assessments

Not applicable to this ISA.

8.9. Medical Resource Utilization and Health Economics

See the IIBD master protocol.

9. Statistical Considerations

The SAP for this ISA will be finalized prior to the first participant first visit. For additional considerations, see the IIBD master protocol.

9.1. Statistical Hypothesis

See the IIBD master protocol.

9.1.1. Multiplicity Adjustment

See the IIBD master protocol.

9.2. Analyses Sets

See the IIBD master protocol.

9.3. Statistical Analyses

9.3.1. General Considerations

See the IIBD master protocol.

9.3.2. Primary Endpoint/Estimand Analysis

See the IIBD master protocol.

9.3.3. Secondary Endpoints/Estimands Analysis

See the IIBD master protocol.

9.3.4. Exploratory Endpoints/Estimands Analysis

ISA-specific exploratory objectives and endpoints are outlined in Section 3.

Details about exploratory estimands and analysis will be specified in the SAP.

9.3.5. Safety Analyses

See the IIBD master protocol.

Details about ISA-specific safety analyses will be specified in the SAP.

9.3.6. Pharmacokinetic Analyses

Plasma concentrations of LY4395089 will be summarized by scheduled time points at respective study visits using descriptive statistics.

Individual concentration versus time data may be presented if appropriate.

9.3.7. Pharmacodynamic Analyses

The exploratory PD endpoints will be summarized descriptively. For details about these endpoints, see Section 3.

Additional analysis of PD endpoints may be described in a biomarker analysis plan separate from the study SAP and reported separately.

9.3.8. Other Analyses

Additional analyses may be specified in the SAP.

9.4. Interim Analyses

See the IIBD master protocol.

9.5. Sample Size Determination

See the IIBD master protocol.

10. Supporting Documentation and Operational Considerations

10.1. Appendix 1: Regulatory, Ethical, and Study Oversight Considerations

See the IIBD master protocol.

10.2. Appendix 2: Clinical Laboratory Tests

The tests detailed in the table below will be performed by the Lilly-designated (central) laboratory or by the local laboratory.

In circumstances where the sponsor approves local laboratory testing in lieu of central laboratory testing (in the table below), the local laboratory must be qualified in accordance with applicable local regulations.

Protocol-specific requirements for inclusion or exclusion of participants are detailed in Section 5 of this ISA, and Section 5 of the IIBD master protocol.

Additional tests may be performed at any time during the study as determined necessary by the investigator or required by local regulations.

Investigators must document their review of the laboratory safety results.

Clinical Laboratory Tests	Comments
Hematology	Assayed by Lilly-designated laboratory.
Erythrocyte count (RBCs - red blood cells)	
Leukocytes (WBCs - white blood cells)	
Hemoglobin	
Hematocrit	Screening: Preferred central, may be performed locally if central result is not available. All other visits: If central result is not available, a prior centrally performed hematocrit may be used if tested within 42 days of the visit.
Mean cell volume (MCV)	
Mean cell hemoglobin (MCH)	
Mean cell hemoglobin concentration (MCHC)	
Absolute neutrophil count (ANC) (segmented and bands) (calculated)	
Platelet count	
Hematology differential	
Absolute count of:	
Neutrophils: segmented	
Neutrophils: bands	
Lymphocytes	
Monocytes	
Eosinophils	
Basophils	
Cell morphology (RBC and WBC)	
Clinical chemistry	Assayed by Lilly-designated laboratory.
Sodium	
Potassium	
Chloride	
Bicarbonate	
Total bilirubin	

Clinical Laboratory Tests	Comments
Direct bilirubin	
Alkaline phosphatase (ALP)	
Alanine aminotransferase (ALT)	
Aspartate aminotransferase (AST)	
Gamma-glutamyl transferase (GGT)	
Blood urea nitrogen (BUN)	
Creatinine	
Creatine kinase (CK)	
Uric acid	At screening only.
Total protein	
Albumin	
Calcium	
Lactate dehydrogenase (LDH)	
Glucose	
Magnesium	
Phosphate	
Glutamate dehydrogenase (GLDH)	
Coagulation	Performed by local laboratory
Prothrombin time-international normalized ratio (PT-INR)	
Hormones (individuals AFAB)	
Follicle-stimulating hormone (FSH)	Assayed by Lilly-designated laboratory. For individuals AFAB only.
Serum pregnancy	Assayed by Lilly-designated laboratory. For IOCBP only. See Sections 8.2.6 and 10.4.1 of the IIBD master protocol.
Urine pregnancy	Evaluated locally. For IOCBP only. See Sections 8.2.6 and 10.4.1 of the IIBD master protocol.
Urine Chemistry	Assayed by Lilly-designated laboratory
Albumin	
Creatinine	
Urinalysis	Assayed by Lilly-designated laboratory. Can be performed at any visit at the discretion of investigator if the participant has symptoms.
Specific gravity	
pH	
Protein	
Glucose	
Ketones	
Bilirubin	
Urobilinogen	
Blood	
Nitrite	
Calcium	

Clinical Laboratory Tests	Comments
Urine leukocyte esterase	
Microscopic examination of sediment	Perform if indicated.
Calculations	Generated by Lilly-designated laboratory
Urinary albumin/creatinine ratio (UACR)	
eGFR (CKD-EPI) calculated using creatinine	
Serology	
Human immunodeficiency virus (HIV) testing	Assayed by Lilly-designated laboratory.
Hepatitis B virus (HBV) testing:	Assayed by Lilly-designated laboratory.
HBV DNA	Performed only for participants who test positive for anti-HBc at screening.
Hepatitis B core total antibody (anti-HBc)	
Hepatitis B surface antigen (HBsAg)	
Hepatitis C virus (HCV) testing:	Assayed by Lilly-designated laboratory.
HCV antibody	
HCV RNA	Performed only for participants who test positive for hepatitis C antibody.
Tuberculosis (TB) testing:	See Sections 8.2.7 and 10.7 of the IIBD master protocol for more information about TB testing.
QuantiFERON®-TB Gold test	Assayed by Lilly-designated laboratory.
T-SPOT®.TB	Tested and evaluated locally. Local laboratory must be qualified by local regulations.
Tuberculin skin test (TST)	Tested and evaluated locally. Local staff must be qualified to administer and interpret the test.
Lipid panel	Assayed by Lilly-designated laboratory.
High-density lipoprotein cholesterol (HDL-C)	
Low-density lipoprotein cholesterol (LDL-C)	
Non-high-density lipoprotein cholesterol (non-HDL-C)	
Triglycerides	
Total cholesterol	
Other	Assayed by Lilly-designated laboratory. Unless specified otherwise, results will not be provided to the investigative sites.
C-reactive protein, high-sensitivity (hsCRP)	Results will be provided to the investigative sites only at screening.
Synthetic glucocorticoid testing	
Intact N-terminal propeptide of type 1 procollagen (P1NP)	
C-terminal telopeptide (CTX1)	
Parathyroid hormone (PTH)	
25-hydroxy-vitamin D	
Bone specific alkaline phosphatase	
Immunoglobulin G (IgG)	
Immunoglobulin A (IgA)	

Clinical Laboratory Tests	Comments
Immunoglobulin M (IgM)	
Stool samples	Assayed by Lilly-designated laboratory.
Stool culture	Additional local testing (for example, for ova and parasites) is allowed at the investigator's discretion.
<i>Clostridioides difficile</i> (<i>C. difficile</i>) testing	Confirmed by a test for <i>C. difficile</i> toxin gene expression.
Fecal calprotectin	Results will be provided to the investigative sites only at screening.
Pharmacokinetic samples	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites.
LY4395089 concentrations	
Pharmacodynamic samples	Assayed by Lilly-designated laboratory. Results will not be provided to the investigative sites.
7AlphaC4	
FGF19	
Genetics	Assayed by Lilly-designated laboratory.
Genetics sample	
Exploratory biomarker samples - stored	
Serum	
Plasma	
Whole blood	
Biopsy tissue	Endoscopic biopsy tissue samples.
Stool supernatant	

10.2.1. Laboratory Samples to Be Obtained at the Time of Systemic Hypersensitivity Reaction

See the IIBD master protocol.

10.3. Appendix 3: Adverse Events and Serious Adverse Events: Definitions and Procedures for Recording, Evaluating, Follow-Up, and Reporting

See the IIBD master protocol.

10.4. Appendix 4: Contraceptive and Barrier Guidance

10.4.1. Definitions

See the IIBD master protocol.

10.4.2. Contraception Guidance for LY4395089/Mirikizumab Co-administration Treatment Group

This section outlines the contraception guidance for participants treated with LY4395089 in the LY4395089/mirikizumab co-administration treatment arm.

10.4.2.1. Individuals Assigned Female at Birth (AFAB)

IOCBP who are completely abstinent as their preferred and usual lifestyle, or exclusively engage in sexual relations with other individual(s) who are AFAB as their preferred and usual lifestyle, must follow the rules in this table.

Must...	Must not...
agree to either remain abstinent or exclusively engage in sexual relations with other individual(s) who are AFAB, and not plan a pregnancy during the study	• use periodic abstinence methods ○ calendar ○ ovulation ○ symptothermal, or ○ post-ovulation • declare abstinence just for the duration of a trial, or • use the withdrawal method

IOCBP who are NOT completely abstinent as their preferred and usual lifestyle, or who do NOT exclusively engage in sexual relations with other individual(s) who are AFAB as their preferred and usual lifestyle, must follow the rules in this table.

Must...
Agree to use 1 highly effective method of contraception together with a barrier method of contraception. These methods of contraception must be used during the study and for at least 10 weeks (approximately 2.5 months) after the last dose of the LY4395089 study intervention, after which the participant must follow the guidance for mirikizumab, described in the IIBD master protocol.

10.4.2.2. Individuals Assigned Male at Birth (AMAB)

The table below describes contraception guidance for individuals AMAB.

Participant Population	Contraception Guidance
All individuals AMAB...	should refrain from sperm donation for the duration of the study and for 10 weeks (approximately 2.5 months) after the last dose of the LY4395089 study intervention, after which the participant must follow the Guidance for mirikizumab, described in the IIBD master protocol.
Individuals AMAB with partner(s) who are IOCBP ^a ...	must • remain abstinent (if this is their preferred and usual lifestyle), or • use condoms and at least 1 additional effective method of contraception for the duration of the study and for 10 weeks (approximately 2.5 months) after the last dose of the LY4395089 study intervention, after which the participant must follow the Guidance for mirikizumab, described in the IIBD master protocol.
Individuals AMAB with partner(s) who are pregnant ^a ...	must • remain abstinent (if this is their preferred and usual lifestyle), or • use condoms for the duration of the study and for 10 weeks (approximately 2.5 months) after the last dose of the LY4395089 study intervention, after which the participant must follow the Guidance for mirikizumab, described in the IIBD master protocol.
Individuals AMAB who exclusively engage in sexual relations with other individual(s) who are AMAB, as their preferred and usual lifestyle...	are not required to use contraception.

Abbreviations: AMAB = assigned male at birth; IOCBP = individuals of childbearing potential.

^a Individuals AMAB who have undergone orchiectomy but not penectomy must use condoms during sex, but the partner who is IOCBP is not required to use an additional form of contraception. Individuals AMAB who have undergone orchiectomy and penectomy are not required to use condoms.

Examples of different **methods of contraception**^a:

Methods	Examples
Highly effective contraception (less than 1% failure rate)	• fallopian tubal sterilization methods other than bilateral salpingectomy (laparoscopic bipolar electrocoagulation, plastic ring application on the uterine tubes, fallopian tube ligation, hysteroscopic sterilization). Note: Bilateral salpingectomy is indicative of permanent sterilization. Please see the INOCBP definition above. • combined (estrogen and progestogen-containing) hormonal contraception associated with inhibition of ovulation: ○ oral ○ intravaginal (vaginal ring) ○ transdermal (for example, contraceptive patch) (only for individuals <198 pounds or 90 kg) • progestogen-only hormonal contraception associated with inhibition of ovulation ○ oral ○ injectable ○ implantable • total abstinence • sexual relationships exclusively between individuals who are assigned the same sex at birth • vasectomy – for individuals AMAB in clinical trials and for the partner of an individual AFAB (if only sexual partner) • fallopian tube implants (if confirmed by hysterosalpingogram), or • intrauterine devices (IUD) with or without hormone-releasing system
Effective contraception	• progestogen-only hormonal contraception, where inhibition of ovulation is not the primary mode of action • penile condom with or without spermicide • vaginal condom with or without spermicide • diaphragm with spermicide • cervical sponge with spermicide, or • cervical cap with spermicide Note: Penile and vaginal condoms should not be used in combination.
Ineffective methods of contraception whether used alone or in any combination	• spermicide alone • periodic abstinence • fertility awareness (calendar method, temperature method, cervical mucus, or symptothermal) • withdrawal • postcoital douche, or • lactational amenorrhea

Abbreviations: AFAB = assigned female at birth; AMAB = assigned male at birth; INOCBP = individuals not of child bearing potential; IUD = intrauterine devices.

^a Ablation (endometrial or uterine) is not considered a form of contraception.

10.4.3. Contraception Guidance for Mirikizumab Treatment Group

See the IIBD master protocol.

10.5. Appendix 5: Genetics

See the IIBD master protocol.

10.6. Appendix 6: Liver Safety: Suggested Actions and Follow-Up Assessments

See the IIBD master protocol.

10.7. Appendix 7: Tuberculosis Testing

See the IIBD master protocol.

10.8. Appendix 8: Examples of Infections That May Be Considered Opportunistic in the Setting of Biologic Therapy

See the IIBD master protocol.

10.9. Appendix 9: Dysplasia Eligibility Decision Algorithm

See the IIBD master protocol.

10.10. Appendix 10: mMS for the Assessment of Ulcerative Colitis Disease Activity

Not applicable to this ISA.

10.11. Appendix 11: Simple Endoscopic Score for Crohn's Disease (SES-CD)

See the IIBD master protocol.

10.12. Appendix 12: Histological Scores

See the IIBD master protocol.

10.13. Appendix 13: Permitted Concomitant Medications

See the IIBD master protocol.

10.14. Appendix 14: Prohibited Concomitant Medications

See the IIBD master protocol for the list of prohibited concomitant medications that applies to all ISAs.

This section outlines additional prohibited medications specific to this ISA. For prohibitions prior to the screening, see Section 5.2.

Drug Class	Prohibited Medication Restrictions
Bone active treatments, including • Oral bisphosphonate • IV bisphosphonate • IV zoledronic acid • IV fluoride • IV strontium • Denosumab • PTH or PTH derivative	Prohibited throughout the duration of the study
Any SERM	Prohibited throughout the duration of the study
Tibolone	Prohibited throughout the duration of the study
Anabolic steroids or testosterone	Prohibited throughout the duration of the study
Systemic hormone replacement therapy	Prohibited throughout the duration of the study Exception: Confirmed hypogonadism treated with stable doses of testosterone replacement therapy for ≥ 2 years prior to screening
Calcitonin	Prohibited throughout the duration of the study
Active vitamin D analogs (25-hydroxy-vitamin D, or 1,25-dihydroxy-vitamin D)	Prohibited throughout the duration of the study Exception: Over the counter vitamin D supplements are acceptable
Other bone active drugs including anticonvulsants	Prohibited throughout the duration of the study Exception: Benzodiazepines
Heparin	Prohibited throughout the duration of the study
Chronic systemic treatment with • Ketoconazole • Androgens • Adrenocorticotrophic hormone (ACTH), • Cinacalcet • Aluminum • Lithium • Protease inhibitors • Gonadotropic releasing hormone agonists	Prohibited throughout the duration of the study
Other FXR agonists	Prohibited throughout the duration of the study
• CYP3A4 inhibitors, strong ○ boceprevir, ceritinib, clarithromycin, cobicistat, conivaptan, idelalisib, itraconazole, josamycin, ketoconazole, lonafarnib, mifepristone, nefazodone, nelfinavir, posaconazole, ribociclib, telithromycin, tucatinib, voriconazole ○ ritonavir (by itself, or in combination with atazanavir, danoprevir, elvitegravir, fosamprenavir, indinavir, lopinavir, nirmatrelvir, paritaprevir and ombitasvir, saquinavir, tipranavir) See Section 5.3 for dietary restrictions, such as grapefruit juice.	Prohibited throughout the duration of the study

Drug Class	Prohibited Medication Restrictions
• CYP3A4 inhibitors, moderate ○ amprenavir, aprepitant, atazanavir, berotralstat, cimetidine, ciprofloxacin, clotrimazole, crizotinib, cyclosporine, darunavir, diltiazem, dronedarone, duvelisib, erythromycin, fedratinib, fluconazole, fluvoxamine, fosnetupitant and palonosetron, imatinib, indinavir, isavuconazole, istradefylline, ledipasvir/sofosbuvir, lefamulin, letemovir, magnolia vine (<i>Schisandra sphenanthera</i>), netupitant, nilotinib, tofisolipam, verapamil, voxelotor	Prohibited throughout the duration of the study
• CYP3A4 inducers, strong ○ aminoglutethimide, apalutamide, carbamazepine (daily dose exceeding 600 mg), enzalutamide, fosphenytoin, ivosidenib, lumacaftor, mitotane, phenobarbital, phenytoin, rifabutin, rifampicin (rifampin), rifapentine, St. John's wort	Prohibited throughout the duration of the study
• CYP3A4 inducers, moderate ○ bosentan, carbamazepine (daily dose 600 mg or lower), cenobamate, dabrafenib, danshen (<i>Salvia miltiorrhiza</i>), efavirenz, elagolix, encorafenib, etravirine, genistein, lopinavir (alone), lorlatinib, modafinil, nafcillin (IV), pentobarbital, primidone, sotorasib, thioridazine, tocilizumab (atlizumab)	Prohibited throughout the duration of the study
• CYP3A4 substrates with narrow therapeutic index ○ alfentanil, alfuzosin, amiodarone, atorvastatin, benzhydrocodone, carbamazepine, colchicine, cyclobenzaprine, cyclosporine, dihydroergotamine, disopyramide, dofetilide, ergotamine, everolimus, fentanyl, indacaterol, isotretinoin, lovastatin, meperidine, methadone, midazolam, oxycodone, pimozide, quetiapine, quinidine, simvastatin, sirolimus, sufentanil, oral tacrolimus (topical use is allowed), telithromycin, tramadol, triazolam	Prohibited throughout the duration of the study
GLP-1 RAs, GLP-1/GIP RAs, GLP-1/GIP/Gcg RAs, or similar therapies	Prohibited from starting new therapies throughout the duration of the study

Abbreviations: ACTH = adrenocorticotrophic hormone; CYP = cytochrome P450; FXR = farnesoid X receptor
 Gcg = glucagon; GIP = glucose-dependent insulinotropic polypeptide; GLP-1 = glucagon-like peptide-1;
 IV = intravenous; PTH = parathyroid hormone; RA = receptor agonist; SERM = selective estrogen receptor modulators.

10.15. Appendix 15: Medical Device Adverse Events (AEs), Adverse Device Effects (ADEs), Serious Adverse Events (SAEs) and Device Deficiencies: Definition and Procedures for Recording, Evaluating, Follow-Up, and Reporting

Not applicable to this ISA.

10.16. Appendix 16: Region- or Country-Specific Requirements

10.16.1. European Union

See the IIBD master protocol.

10.16.1.1. Germany

See the IIBD master protocol.

10.16.2. Brazil

See the IIBD master protocol.

10.16.3. Canada

See the IIBD master protocol for additional protocol changes applicable for participants in study sites in Canada.

The revised text described below shows the changes applicable to participants at study sites in Canada. Deletions are identified by strikethrough format and additions by underlined text.

10.4.2.1. Individuals Assigned Female at Birth (AFAB)

IOCBP who are NOT completely abstinent as their preferred and usual lifestyle, or who do NOT exclusively engage in sexual relations with other individual(s) who are AFAB as their preferred and usual lifestyle, must follow the rules in this table.

Must...
Agree to use 1 highly effective method of contraception together with a barrier method of contraception. These methods of contraception must be used during the study and for at least 40<u>12</u> weeks (approximately 2.5<u>3</u> months) after the last dose of the LY4395089 study intervention, after which the participant must follow the guidance for mirikizumab, described in the IIBD master protocol.

10.4.2.2. Individuals Assigned Male at Birth (AMAB)

The table below describes contraception guidance for individuals AMAB.

Participant Population	Contraception Guidance
All individuals AMAB...	should refrain from sperm donation for the duration of the study and for 40<u>12</u> weeks (approximately 2.5<u>3</u> months) after the last dose of the LY4395089 study intervention, after which the participant must follow the Guidance for mirikizumab, described in the IIBD master protocol.
Individuals AMAB with partner(s) who are IOCBP ^a ...	must • remain abstinent (if this is their preferred and usual lifestyle), or • use condoms and at least 1 additional effective method of

Participant Population	Contraception Guidance
	contraception for the duration of the study and for 40 <u>12</u> weeks (approximately 2-5 <u>3</u> months) after the last dose of the LY4395089 study intervention, after which the participant must follow the Guidance for mirikizumab, described in the IIBD master protocol.
Individuals AMAB with partner(s) who are pregnant ^a ...	must • remain abstinent (if this is their preferred and usual lifestyle), or • use condoms for the duration of the study and for 40<u>12</u> weeks (approximately 2-5<u>3</u> months) after the last dose of the LY4395089 study intervention, after which the participant must follow the Guidance for mirikizumab, described in the IIBD master protocol.
Individuals AMAB who exclusively engage in sexual relations with other individual(s) who are AMAB, as their preferred and usual lifestyle...	are not required to use contraception.

Abbreviations: AMAB = assigned male at birth; IOCBP = individuals of childbearing potential.

- ^a Individuals AMAB who have undergone orchiectomy but not penectomy must use condoms during sex, but the partner who is IOCBP is not required to use an additional form of contraception. Individuals AMAB who have undergone orchiectomy and penectomy are not required to use condoms.

10.16.4. United States

See the IIBD master protocol.

10.17. Appendix 17: Provisions for Changes in Study Conduct during Exceptional Circumstances

See the IIBD master protocol. For the list of named biomarkers, see Section [8.5](#) of this ISA.

10.18. Appendix 18: Abbreviations and Definitions

See the IIBD master protocol for full list of abbreviations and definitions.

Term	Definition
AE	adverse event
AESI	adverse event of special interest
AFAB	assigned female at birth
AMAB	assigned male at birth
ALP	alkaline phosphatase
ALT	alanine aminotransferase
AST	aspartate aminotransferase
authorized IMP	<i>Applicable to the EU only:</i> a medicinal product authorized in accordance with Regulation (EC) No 726/2004 or in any Member State concerned in accordance with Directive 2001/83/EC, irrespective of changes to the labelling of the medicinal product, which is used as an investigational medicinal product
authorized AxMP	<i>Applicable to the EU only:</i> a medicinal product authorized in accordance with Regulation (EC) No 726/2004, or in any Member State concerned in accordance with Directive 2001/83/EC, irrespective of changes to the labelling of the medicinal product, which is used as an auxiliary medicinal product
AxMP	auxiliary medicinal product. See also NIMP. A medicinal product used for the needs of a clinical trial as described in the protocol, but not as an investigational medicinal product. Examples include rescue medication, challenge agents, agents to assess endpoints in the clinical trial, or background treatment. AxMP does not include investigational medicinal product (IMP) or concomitant medications. Concomitant medications are medications unrelated to the clinical trial and not relevant for the design of the clinical trial
BMD	bone mineral density
CD	Crohn's disease
CKD-EPI	Chronic Kidney Disease Epidemiology Collaboration
complaint	A complaint is any written, electronic, or oral communication that alleges deficiencies related to the identity, quality, purity, durability, reliability, safety or effectiveness, or performance of a drug or drug delivery system.
compliance	Adherence to all study-related, good clinical practice (GCP), and applicable regulatory requirements.
CRF	case report form; a printed, optical, or electronic document designed to record all of the protocol-required information to be reported to the sponsor for each trial participant.

CYP	cytochrome P450
DMC	Data monitoring committee. A data monitoring committee, or data monitoring board (DMB) is a group of independent scientists who are appointed to monitor the safety and scientific integrity of a human research intervention, and to make recommendations to the sponsor regarding the stopping of a study for efficacy, or for harm, or for futility. The composition of the committee is dependent upon the scientific skills and knowledge required for monitoring the particular study.
DXA	dual-energy x-ray absorptiometry
ECG	electrocardiogram
eGFR	estimated glomerular filtration rate
enroll	The act of assigning a participant to a treatment. Participants who are enrolled in the study are those who have been assigned to a treatment.
enter	Participants entered into a study are those who sign the informed consent form directly or through their legally acceptable representatives.
FXR	farnesoid X receptor
Gcg	glucagon
GCP	good clinical practice
GGT	gamma-glutamyl transferase
GIP	glucose-dependent insulinotropic polypeptide
GLP-1	glucagon-like peptide-1
HDL-C	high density lipoprotein-C
IB	Investigator's Brochure
ICF	informed consent form
IL	interleukin
informed consent	A process by which a participant voluntarily confirms their willingness to participate in a particular study, after having been informed of all aspects of the study that are relevant to the participant's decision to participate. Informed consent is documented by means of a written, signed and dated informed consent form.
interim analysis	An interim analysis is an analysis of clinical study data, separated into treatment groups, that is conducted before the final reporting database is created/locked.
investigational product	A pharmaceutical form of an active ingredient or placebo being tested or used as a reference in a clinical trial, including products already on the market when used or assembled (formulated or packaged) in a way different from the authorized form, or marketed products used for an unauthorized indication, or marketed products used to gain further information about the authorized form. See also "IMP."

INOCBP	individuals not of child bearing potential
IOCBP	individuals of child bearing potential
ISA	indication-specific appendix
IWRS	interactive web-response system
LDL-C	low density lipoprotein-C
MASH	metabolic dysfunction-associated steatohepatitis
participant	Equivalent to CDISC term “subject”: an individual who participates in a clinical trial, either as recipient of an investigational medicinal product or as a control.
PD	pharmacodynamics
PK	pharmacokinetics
QTc	corrected QT interval
RA	receptor agonist
SAE	serious adverse event
SAP	statistical analysis plan
SC	subcutaneous
screen	The act of determining if an individual meets minimum requirements to become part of a pool of potential candidates for participation in a clinical study.
TNBS	2,4,6-trinitrobenzene sulfonic acid
UC	ulcerative colitis
ULN	upper limits of normal

11. References

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